

ONTARIO
SUPERIOR COURT OF JUSTICE

BETWEEN:	)	
	)	
REGINA MARIE MACEACHERN	)	
	)	R. M. Gratsias, for the Plaintiff
Plaintiff	)	
	)	
– and –	)	
	)	
TFGI (HUNTSVILLE) INC. o/a	)	Not Appearing
BRENDALE SQUARE PLAZA	)	
	)	
Defendant	)	
	)	
	)	
	)	HEARD: January 4, 2018

HEALEY J:

REASONS FOR JUDGMENT

- [1] This is a motion for judgment pursuant to Rule 19.05.
- [2] The defendant did not serve a defence and was noted in default on June 19, 2013.
- [3] Three affidavits were filed for this motion for judgment, each from lawyers from the law firm retained by the plaintiff. The initial affidavit summarizes some of the medical records and details the plaintiff's physical injuries resulting from the fall. However, it also purports to provide evidence of the plaintiff's economic losses and past and future care costs, evidence which is better suited to be provided by the plaintiff, if possible. For this reason, this Court ordered that the motion was to proceed with oral evidence.
- [4] The last two affidavits contained in the supplementary record are also from lawyers and simply supplement the evidence by attaching reports and other documents such as OHIP records.

Liability

- [5] Liability is established by way of deemed admission of the facts set out in the Statement of Claim pursuant to Rule 19.02(1)(a). Those facts establish that on February 24, 2011 the plaintiff slipped on an icy parking lot on the defendant's property, which parking lot had not been cleared on the date of the fall. The plaintiff was walking carefully at the time. The plaintiff supplemented this in her oral testimony by indicating that the parking lot in question is located in Huntsville, that it was snowing heavily at the time, and that she was wearing skidoo boots. She knew that there were icy patches in the parking lot the day before when she had attended at work. She was looking down and trying to avoid stepping on any icy patches, attempting to see through the snow layered on the ground. The parking lot had also not been sanded. After she slipped her purse and cell phone were beyond her reach, and she had to rely on the goodwill of a passersby to contact an ambulance, which took her to the hospital in Huntsville.
- [6] This Court finds that the icy, unsafe condition on the defendant's property was caused by the negligence of the corporate defendant, who, on the date in question, failed to meet the standard of care required of a property owner under the Occupiers' Liability Act, R.S.O. 1990, c. O.2, as detailed in paragraph 5 of the Statement of Claim.

General Damages

- [7] The plaintiff was 62 years old at the time of the slip and fall incident. Paragraph 6 of the Statement of Claim details the physical injuries caused by the fall, which I find occurred as an immediate and proximate result of the defendant's negligence. These physical injuries include a tibial plateau fracture to her left knee, and a general tearing and straining of the muscles, ligaments, and tendons throughout her body. The ambulance call report from that date notes that she was unable to weightbear and pain increased to 10 out of 10 when she attempted to move.
- [8] The attendants noted that she reported a medical history including hypertension and arthritis. The CT of the plaintiff's left knee from the date of the fall notes that degenerative changes are present throughout the knee with moderate to marked joint space narrowing in the medial compartment and moderate osteophytes throughout the knee.
- [9] A history and physical examination taken by Dr. Melanie Mark at Huntsville District Memorial Hospital on February 25, 2011 indicates that the plaintiff's past medical history included obstructive sleep apnea, degenerative disc disease, previous knee dislocations, osteoarthritis, hallux valgus, hypertension and hypothyroidism.
- [10] The plaintiff explained that she believes that some of her pre-existing back and knee pain can be attributed to the fact that she was the primary caregiver for her mother before she passed away, which involved lifting, bending and other movements necessary to care for her mother's needs.

- [11] The plaintiff was transferred to Royal Victoria Hospital in Barrie approximately one week after the fall for repair of the left tibial plateau fracture, performed by Dr. Casses, which included a bone graft and the insertion of three screws. She was discharged two days later, but returned to the Huntsville District Memorial Hospital the next day due to being unable to cope with navigating her home in Burk's Falls with a wheelchair. A further history taken by Dr. Robert Riddle on the date of her readmission on March 6, 2011 indicates that her main problem prior to the fall was that she had osteoarthritis of both knees and was having difficulty with activities of daily living. She was taking Celebrex for relief. She also had a history of obesity.
- [12] The plaintiff remained in hospital in Huntsville until her discharge on March 28, 2011. She had physical therapy throughout her stay, and was followed by Dr. Casses for cast changes and follow-up. In addition to her regular medications, she was also given a prescription for Percocet, Enoxaparin and Fucidin ointment.
- [13] After surgery the plaintiff spent the next three months in a wheelchair, and was casted throughout that time. She was completely non-weightbearing for over five months and described that she endured significant pain throughout. As a result of being seated for that extended period, the plaintiff found that she developed significant back pain. During the time that she was in a wheelchair she was unable to access the upstairs of her home, and therefore slept on a hospital bed in the living room and used a commode. Community Care Access Centre ("CCAC") attended her home daily for house cleaning and personal care for approximately 9 months, and provided physiotherapy for approximately 3 months.
- [14] The plaintiff described that her mobility has now changed dramatically. She suffers from constant pain in both knees. Overuse of her right leg has led to increased right knee pain compared to her pre-accident status. She attended an orthopedic specialist in Ottawa in April, 2016 who suggested cortisone shots in her right knee, which she found did not help. She described that the soft tissues on the inside of her left knee were damaged in the fall, and she continues to feel piercing pain on the interior of the knee which feels like electric shocks. She has progressed from walking with a walker to using a cane whenever she leaves her son's apartment, where she now resides.
- [15] A note from her former family doctor, Dr. Schuurman, confirmed that during her initial recovery period the plaintiff was putting increased weight on her right leg and this led to worsening right knee pain. She saw Dr. Galvin in consultation, who recommended injections. The evidence appears to be that she received those injections. Dr. Schuurman also noted that the plaintiff reported an increase in her low back pain after the 2011 fracture. Dr. Schuurman's note also states that as of October, 2011 Dr. Casses was concerned that she would require left knee replacement surgery in the future. Dr. Schuurman stopped treating the plaintiff as of December, 2014.
- [16] Although she attempted to do some vacuuming and cleaning at her home in Burk's Falls after CCAC funding ended, she was unable to do so without great difficulty, sometimes attempting to access areas to be cleaned by sitting on a chair.

- [17] Although the plaintiff returned to her matrimonial home after her hospital discharge, the change in her mobility created a significant change in the dynamics of her marriage. It is her evidence that her husband resented that he had to look after her, which was a change in roles. They are currently separated and she has lived with her son in Ottawa since late 2015.
- [18] The plaintiff's son is 33 years of age. The plaintiff describes that he has put his own life on hold to stay in Ottawa so that she can be close to a hospital and he can assist her. She is now far more reliant on him than she was before the fracture. He does the shopping and carries things for her because of how unstable she feels when attempting to hold things in her hands. He carries items upstairs for her, because other than small bags, she is unable to carry much weight.
- [19] Although the plaintiff suffered from osteoarthritis in her left knee prior to the fall, there is evidence that she was living a robustly active life which was minimally impacted, if at all, by that condition. She was employed on a part-time basis at Fabricland, where she had to assist with unloading and carrying bolts of fabric and was on her feet for hours at a time. Her pastimes included gardening, during which she would kneel on her knees, weaving with a floor loom that required her to use her legs to operate treddles, cooking at a high skill level, in which she described standing for long periods of time while she did preparation work, and being involved in renovations to both her current home in Burk's Falls and previous residences. She and her husband intended to open a bed and breakfast at their farm in Burk's Falls. She described that she did not have difficulty navigating stairs, and never walked with a cane or other assistance. The plaintiff did all of her own housecleaning, which she described as being to a high standard. She described that she did everything in her home related to housekeeping, including climbing ladders to wash windows, grocery shopping, cooking and taking laundry to the laundromat because she and her husband had dismantled the laundry room in the basement just prior to her accident. She regularly enjoyed walking her dogs, even over uneven ground and rolling fields. She described that she would walk quickly. Another hobby was to shop through flea markets and antique stores. She has not done so since the accident because walking irritates her knees and back. She has not been able to resume any of these previous recreational pastimes and hobbies.
- [20] Before the fracture she had received a CPAP machine for her sleep apnea and had begun losing weight. Some of that weight has been regained since her injury.
- [21] In terms of self-care, she is able to shower, but with difficulty. She now has to sit on a chair in the shower covered with a plastic bag. Any non-flat surfaces create difficulty for her knees, and cause her to feel insecure. Her left knee remains sore on the inside, to the touch. She is unable to bend her right knee sufficiently to clip her own nails, and is on a waitlist for a foot clinic in Ottawa. She is now required to wear compression stockings all of the time because she sits more frequently. In the past she sometimes wore them at work, but not regularly.

- [22] She now lives in constant fear that she will fall and sustain another fracture. She has to sit frequently due to pain in her back and knees. The cooking that she does now is far less complicated because she cannot stand to do the preparation work. She is in pain 99.9% of the time, wakes frequently at night, and describes that her life is significantly less enjoyable. Long car rides cause greater discomfort. Although she has been able to resume some form of work from time to time, this has been sporadic, and has been complicated by additional health problems, which will be detailed later.
- [23] This evidence leads this Court to the conclusion that the plaintiff suffered a significant injury on February 24, 2011, which developed further sequelae including increased strain on her right knee and low back. Although there is evidence that there were prior degenerative changes in both knees and her back, there is convincing evidence that these conditions were being managed medically and that the plaintiff was not displaying any significant lack of function or mobility arising from those conditions prior to the date of her injury.
- [24] The injury has impacted all areas of her life, and in particular has significantly reduced her independence. She is now reliant on her son for most of the more strenuous aspects of daily living. Although she mobilizes within his apartment without a cane by holding on to furniture as necessary, when she leaves the apartment she cannot do so without an assistive device such as her cane or walker.
- [25] All of her injuries have dramatically altered her level of mobility and independence, prevented her from pursuing her previous recreational pastimes, caused or contributed to the breakdown of her marriage and plans for her future income through operation of a bed and breakfast, cause ongoing significant pain, and have reduced her enjoyment of life.
- [26] I found the plaintiff to be a credible witness whose testimony was candid, straightforward, and believable.
- [27] In terms of assessing her general damages, plaintiff's counsel provided the court with four cases in which the plaintiff suffered fractures of the lower limb. In *Cartner v. Burlington (City)*, 2008 CanLII 37900 (ON SC) the 51-year-old plaintiff slipped and fell on an icy sidewalk and suffered a severe fracture of her right femur. The fracture was surgically treated with a steel plate and screws. Similar to Mrs. Maceachern, the plaintiff developed stiffness, swelling and pain in both knees from lack of mobility and inability to weightbear. She continued to be plagued by severe pain in both of her knees. Also, her right knee, which have borne the brunt of her weight in the months following her surgery when she had not been able to weightbear, had pain all of the time. The plaintiff underwent a total right knee replacement four years after her initial injury due to a significant acceleration of osteoarthritis in both knees, and the replacement of her left knee was anticipated. She was physically incapable of continuing to earn her livelihood as a cashier. Her non-pecuniary general damages for pain and suffering and loss of enjoyment of life were assessed to be \$120,000. This converts to approximately \$138,090 in 2018.

- [28] In *Bannon v. Thunder Bay (City)*, 2002 CarswellOnt 2832 a 47-year-old plaintiff suffered a fracture of her right femur. Like Mrs. Maceachern, the plaintiff had a prolonged hospital admission. She had pre-existing arthritic changes in her neck and back, and arthritis in both knees. She had had a right knee replacement four years before her fracture. Like Mrs. Maceachern, she began to use her left leg more to compensate for the weak right knee, and as a result began to develop discomfort in the left knee. The court assessed her damages at \$85,000, and refused to reduce that amount by 25% on the basis of the defendant's argument that the plaintiff failed to mitigate her damages by losing weight. The evidence was that Mrs. Bannon had struggled to sustain weight loss even before the fracture, and had a family history of obesity. In 2018 dollars, the sum of \$85,000 is approximately \$109,956.
- [29] The third case presented on behalf of the plaintiff is *Ivens v. Lesperance*, 2012 ONSC 4280. The plaintiff sustained a fractured left knee in a motor vehicle accident. Mulligan, J., who determined a threshold motion pursuant to s. 267.5 of the *Insurance Act*, R.S.O. 1990, c. I.8, found that the medical evidence established that he sustained a permanent serious impairment of important physical functions. The plaintiff presented evidence that his knee fracture and contusion resulted in sleep, mood and chronic pain disorder and function. The jury awarded the plaintiff \$100,000 in general damages, or \$107,711 in current dollars.
- [30] The final case provided by plaintiff's counsel is *Iacolucci v. Fernbrook Homes (Brooklin) Ltd.*, 2006 CarswellOnt 9398 (S.C.J.). The plaintiff was 53 years old when she fell off her front porch and sustained a tibial fracture of her right leg, which required surgical intervention. Two years later, she required a total knee replacement. The evidence was that a third surgery may be necessary, but was unlikely to produce a satisfactory result. Like Mrs. Maceachern, the plaintiff resumed working but had difficulties and needed accommodation for functions that required standing for long periods of time. The court awarded general damages of \$175,000, or \$210,417 in current dollars.
- [31] As previously indicated, the plaintiff has suffered further unrelated health complications post-fracture. In August 2014 she suffered from an obstructed kidney stone causing acute kidney injury. In 2015 she went to the Arctic, a location where she had lived during childhood with her parents. While there, suffered a chest infection and was subsequently followed for severe asthma. In March, 2016 she fell on ice, dislocated her shoulder and suffered a right rotator cuff tear. She saw a rheumatologist, Dr. Brian Boate, in Ottawa in April, 2016. His consultation note indicates that her most painful region is her low back, followed by knees, neck and both 1st CMC joints (hand). The note also indicates that she has a history of bilateral carpal tunnel syndrome documented by a neurologist in Guelph, along with the right ulnar nerve entrapment for which repeat nerve conduction tests were pending. She was having physiotherapy at the time for the rotator cuff tear.
- [32] Dr. Boate's consultation note indicates that the plaintiff has tried corticosteroid to several joints with noted improvement. She had Synvisc injections to her knees, which were also beneficial. She has taken Celebrex for years with noted improvement, finding a larger

dose to be more effective. She supplements this with Tylenol Extra Strength. On examination she was found to have decreased range of movement in the neck as a result of cervical spondylosis, a condition that had been seen on an MRI of the neck and lumbar spine done on July 29, 2014 in Guelph. She had osteoarthritis involving both 1st CMC joints. She had osteoarthritis involving her knees. Recent knee x-rays showed severe involvement of the medial compartment bilaterally. She had a reasonable range of movement in the lumbar spine. It was his assessment that she had the following problems:

- (1) Generalized osteoarthritis with severe involvement of the knees followed by neck, lumbar spine and hands;
- (2) Right rotator cuff tear as a result of her recent fall in 2016;
- (3) Bilateral carpal tunnel syndrome more on the right than left.

- [33] Dr. Boate did not change any of her medications but injected each of her knees with corticosteroid. The plaintiff indicated in her testimony that Dr. Boate discussed knee replacement with her and told her that she was too young. She also stated that she did not return to him for further treatment, as she could not walk for over a month after the injections due to pain.
- [34] The evidence presented leads to the conclusion that the injury suffered by the plaintiff exacerbated and possibly accelerated her arthritic and degenerative changes in her knees and back. Despite a prolonged period of hospitalization and subsequent physiotherapy and other treatment modalities, her limb continues to have pain and reduced function. Back and bilateral knee pain continues, and causes instability.
- [35] While recognizing that the plaintiff had pre-existing arthritis in the areas which are currently causing her the major problems, there is no evidence from the defence to lead to the conclusion that her current level of mobility was a foregone conclusion at this age. Although there is evidence that osteoarthritic and degenerative changes were present in both knees and lower and upper back before February 2011, I find that the plaintiff has proven on a balance of probability that those health conditions were not limiting her mobility nor causing pain anywhere near her current problems. The Court will not engage in speculative conclusions about where she may have been today had the accident not occurred in the absence of any evidence led by the defence. Further, the evidence points to the conclusion that the plaintiff has pursued treatment actively and attempted to mitigate her general damages.
- [36] Taking all of these circumstances into account, I find that a fair award of general damages in this case is \$115,000.

Loss of Earnings

- [37] The Statement of Claim pleads that the plaintiff has suffered and will continue to suffer from an inability to work, a loss of time from work, and resulting loss of income. Her ability to compete in the marketplace has been and will be diminished.
- [38] The plaintiff has filed a report from PricewaterhouseCoopers LLP which purports to calculate the plaintiff's future economic loss and present value housekeeping costs as a result of the slip and fall incident. I am disregarding this report in its entirety for three reasons. There is no evidence of compliance with Rule 53.03, and in particular, a Form 53 acknowledgment is lacking. Further, the calculation of income loss is based on faulty assumptions not supported by the evidence. Last, PricewaterhouseCoopers LLP is not in a position to opine on the plaintiff's need for future medical or housekeeping care, and the plaintiff's lawyers have not filed medical documents supporting the necessity of such expenditures or their duration.
- [39] The Court has no information about the plaintiff's education or employment history, other than that during the course of her testimony she referenced a university degree.
- [40] At the time of the incident the plaintiff was employed as a sales clerk at Fabricland. Her income from this job historically had been \$17,648 in 2008, \$18,353 in 2009, and \$16,442 in 2010. The affidavit evidence of one of her lawyers, Sancia Pinto, states that prior to the incident the plaintiff was able to work multiple shifts a week, full days and weekends.
- [41] In 2011 her total income was \$13,228 comprised of employment income, employment insurance benefits and a negligible amount of commission income.
- [42] Following the fracture, the plaintiff's evidence is that she returned to work at Fabricland in 2012 once her employment insurance benefits ran out. Her evidence is that she attempted to find more sedentary work, unsuccessfully, and so continued to work at Fabricland with reduced hours. She could no longer lift heavy bolts of fabric or take part in the receiving of heavy material. She continued to work there for the next three years. Her income was \$16,056 in 2012, \$15,892 in 2013 and \$13,041 in 2014.
- [43] Although her evidence was that she was already beginning to look for a different job before the slip and fall incident, there is insufficient evidence from which to conclude that, had the accident not occurred, those efforts would have been successful and have led to higher income.
- [44] She did not leave Fabricland due to pain or limitations. Instead, she stopped working at Fabricland when she went to the Arctic to attend a funeral in late January, 2015. Her employer was holding her job for her, but once she was away from Ontario she determined that she wanted a better job, and wanted to separate from her husband. She remained in the Arctic. She attempted to obtain a job with government agencies, unsuccessfully, testifying that in her experience individuals with Inuit heritage were given preference for those jobs.

- [45] On her return from the Arctic in late 2015 she went to Ottawa to live with her son. She attempted to obtain a job with Inuit Affairs in Ottawa. She registered with an employment agency, knowing that she needed to obtain a sedentary job. She got a job with Elections Canada at a call center at the time of the federal election, but that contract terminated at the end of December, 2015. Her 2015 income tax return shows employment income of \$6,179 and employment insurance benefits of \$7,439 for a total of \$13,618.
- [46] The plaintiff's next period of employment spanned from November, 2016 to May, 2017. She became a residential worker in a live-in situation through an Inuit organization. She worked 12 hour weekend shifts, earning \$17 an hour. The funding for the program ran out in May, 2017. Otherwise, she did not obtain any other work in 2017. No records were filed with respect to the plaintiff's 2016 or 2017 income, although this would have greatly assisted the Court. Using rough calculations, assuming the plaintiff worked 2 months or 96 hours in 2016 and 5 months or 240 hours in 2017, her gross income from this source would be approximately \$1,632 in 2016 and \$4,080 in 2017.
- [47] It is the plaintiff's evidence that she currently cannot be employed in retail as she is unable to stand for any length of time. As her mobility has become worse over time, her evidence is that she would need a job with comfortable seating, but that even sitting for prolonged periods aggravates her back pain. She is hampered in obtaining a job in Ottawa because she is not fluently bilingual, and she believes that she is at a disadvantage because most of her competition is younger, "svelter", and not required to use a cane. Although she expressed an interest in opening a small arts and crafts store in the Guelph area, potentially, it is obvious that there are numerous physical challenges to her doing so.
- [48] With respect to future loss or damage, the plaintiff needs to prove that there is a real and substantial possibility of such loss or damage occurring.
- [49] The plaintiff is currently 67 years old. She is disabled by her knee and back pain and lack of mobility. Although she may have been able to work in the years following the incident, the medical evidence, together with the plaintiff's testimony, demonstrates that she has deteriorated in the past two years. I find that a retail job of the sort that she had at Fabricland is now beyond her physical capabilities. However, I also find that, had the accident not occurred, given the constellation of pre-existing health difficulties in combination with those that have arisen since the incident, the plaintiff did not and does not have a reasonable expectation of a prolonged work life. Although the plaintiff obviously has a keen and active mind, it is unlikely that she would have remained employed, at the most, beyond the next five years. A reasonable retirement age in the circumstances would have been, at the most generous, age 72.
- [50] Using the plaintiff's average income from the three years preceding the incident of \$17,481 as the benchmark for what her earnings could have been had the accident not occurred, I calculate the past loss of income to the end of 2017, coinciding ostensibly with the date of the hearing, to be \$44,820.

- [51] The present value of the plaintiff's loss of income from January 3, 2018 to July 14, 2022 when the plaintiff will turn 72, will require the assistance of an individual with expertise in valuing economic loss. In addition to the foregoing assumption of retirement age, the sum of \$17,481, adjusted for inflation, should be used to calculate the future loss of income.

Attendant Care

- [52] The plaintiff's counsel in her submissions sought an award for future attendant care in the amount of \$3,000 per month for life. Other than the plaintiff's evidence of her son's assistance with shopping and carrying, there is insufficient evidence to form the basis of such an award, and in particular no medical evidence to substantiate such a need.

Housekeeping

- [53] The plaintiff's evidence provides the basis for this award. It is obvious that as a direct result of the incident she is unable to carry out daily housekeeping tasks to the extent that she could before, if at all, and the modest sum of \$100 per week is reasonable. However, it is also unreasonable, given the constellation of medical issues, to expect the plaintiff to live independently for her anticipated life expectancy. There is the additional problem that the plaintiff has not led evidence as to whether she can anticipate a normal life expectancy, given some of her pre-and post-incident health conditions. I find that it is reasonable to expect that at some point the plaintiff will need to move into an assisted living arrangement where housekeeping will be provided. Recognizing that I am hampered by lack of evidence, I place this at age 75. Again, assistance will be required for the Court by way of a calculation of the present value of a housekeeping cost of \$100 per week to age 75.

Medical/Rehabilitation Costs

- [54] The plaintiff's counsel in her submissions requested the sum of \$3,000 annually for 20 years for medical and rehabilitation costs. I have no evidence as to what the plaintiff's medication costs have been or will be in the future, what assistive devices may be required and how often they will be replaced, or any out-of-pocket costs for medical or rehabilitation costs. Further, the only evidence of current rehabilitation is physiotherapy, which is being carried out strictly for an unrelated injury. While medications are being taken, I have no evidence of associated cost. Again, there is insufficient evidence to form the basis of such an award.

OHIP

- [55] The plaintiff seeks an award of \$36,672.20 to satisfy OHIP's subrogated claim. That amount shall be awarded. She also seeks a further unquantified amount on behalf of OHIP for the future cost of surgical knee replacements.
- [56] Although there is a suggestion that knee replacements have been discussed with at least one physician, there is no medical report before the Court to assist in reaching the

conclusion that there is a real and substantial possibility that she will have to undergo this future surgery.

Plaintiff's Out-of-Pocket Expenses

- [57] The plaintiff has put forward evidence of debts owed to her son for out-of-pocket costs to travel from Ottawa to Barrie attend court hearings, including vehicle rental and hotel costs. Supporting invoices were included. I award the plaintiff the sum of \$1,499.89 for this past pecuniary loss.

Costs

- [58] The plaintiff has provided a costs outline for all steps in the action, including disbursements. The amount sought of \$20,067.20 has been calculated on a partial indemnity basis. Given that this was an undefended action, the amount of time spent on the file, calculated at 59 hours including the appearance on the motion for judgment, is excessive particularly given the gaps existing in the evidence. I am prepared to grant a cost award using 2/3 of that time, or 40 hours given that further work will be necessary.
- [59] Undoubtedly the amount of disbursements will increase given the Court's direction to provide expert evidence of present value calculations. Accordingly, the final cost award will be determined by this Court in the future.

Prejudgment Interest

- [60] Prejudgment interest will be awarded in accordance with the Rules on all qualifying awards.

Conclusion

Counsel is directed to provide a draft judgment for this Court's consideration, to incorporate the awards referenced in these Reasons for Judgment, together with the present value calculations of future pecuniary loss as directed. Evidence of these calculations is also required. Prejudgment interest, together with the calculation thereof, shall also be provided. Proof of any further disbursements incurred is also required, in order that the Court can finalize the cost award. This material is to be directed to the office of the judicial assistants in Barrie, to my attention.

HEALEY J.

Released: January 16, 2018